

Undergraduate Medicine

Orthopedics Study Pack

CharaNas Medicine — Specialty Notes

Expanded notes with original schematic figures for revision. These are educational drawings inspired by standard undergraduate teaching themes, not copied textbook plates. Always follow your faculty curriculum and latest guidelines.

Section	Focus
1	Fracture principles
2	Compartment syndrome & emergencies
3	Soft tissue & Ottawa rules
4	Pediatric & elective themes
5	Checklist & books
6	Quick pearls from these notes
7	Recommended book sources

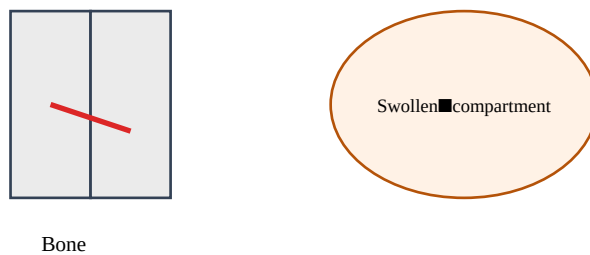
1. Fracture principles

Describe fractures: bone, open vs closed, location, pattern (transverse/oblique/spiral/comminuted), displacement/angulation, joint involvement.

ATLS first for major trauma. Open fractures need antibiotics, tetanus status, and urgent ortho. Immobilize and assess neurovascular status before and after reduction/splinting.

Colles fracture: FOOSH → distal radius dorsal angulation. Hip fracture in elderly: shortened externally rotated limb.

Figure: Fracture & compartment idea (schematic)



Rising pressure → ischemia; pain out of proportion is a red flag

Schematic figure for learning — verify details in your recommended textbooks.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about orthopedics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

2. Compartment syndrome & emergencies

Compartment syndrome: pain out of proportion, pain on passive stretch, tense compartments — pulses may still be present. Urgent fasciotomy pathway.

Fat embolism after long-bone fracture: respiratory distress, neurologic change, petechial rash.

Septic arthritis: hot swollen joint, inability to bear weight/fever — urgent aspiration and antibiotics after cultures; joint emergency.

Revision prompts

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- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

3. Soft tissue & Ottawa rules

Ottawa ankle/knee rules reduce unnecessary radiographs after sprain mechanisms while catching clinically important fractures.

Sprain grades and RICE/PEACE & LOVE style early care are common teaching frameworks; unstable injuries need specialty review.

Back pain red flags: trauma, cancer history, infection signs, saddle anesthesia, bowel/bladder change, progressive neurology — consider cauda equina.

Revision prompts

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- Name 2 investigations and 2 initial management steps you would consider first.

4. Pediatric & elective themes

Children: greenstick fractures, growth plate (Salter–Harris) injuries — growth disturbance risk. Non-accidental injury must be considered when history/injury mismatch.

Osteoarthritis vs inflammatory arthritis patterns differ in morning stiffness duration, systemic features, and erosions.

Pre-op optimization: VTE risk, infection risk, bone health, and rehab planning matter as much as the operation.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about orthopedics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

5. Checklist & books

Checklist: look/feel/move, neurovascular exam, joint above and below, appropriate X-rays (two views), analgesia, immobilize, escalate emergencies early.

Books: Apley's System of Orthopaedics; Campbell's; Bailey & Love ortho chapters.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about orthopedics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

6. Quick pearls

High-yield reminders packaged with this specialty in CharaNas:

1. Compartment syndrome: pain out of proportion — emergency fasciotomy pathway.
2. Colles fracture: FOOSH → distal radius dorsal angulation.
3. Ottawa ankle rules guide X-ray after sprain.
4. Hip fracture: shortened externally rotated limb in elderly fall.
5. Fat embolism after long-bone fracture: hypoxia, neuro, petechiae.
 - Link symptoms → anatomy/physiology → investigation → first management step.
 - Write one OSCE-style explanation for a common presentation in this specialty.
 - After reading, do the Short MCQ and Case-based Question for active recall.

7. Recommended book sources

Standard undergraduate references commonly used internationally. Use the edition your college recommends. Figures in commercial textbooks are copyrighted; use library/licensed access for full plates and photographs.

#	Reference
1	Apley's System of Orthopaedics and Fractures
2	Campbell's Operative Orthopaedics
3	Bailey & Love — Orthopaedics chapters

How to study with this PDF: read one section → sketch the figure from memory → answer the MCQs/cases → review mistakes → revisit the matching section.

Disclaimer: educational aid only; not a substitute for clinical guidelines, faculty teaching, or licensed textbooks.